

C815

# Quality and Performance Management and Methods

Armored Drill

8 concepts · 12 items · 4 transfer items

**What this is.** Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

**Source.** Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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# How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

|                             |                                           |                        |                                  |
|-----------------------------|-------------------------------------------|------------------------|----------------------------------|
| <b>Target word</b><br>_____ | <b>The one fact that decides</b><br>_____ | <b>Answer</b><br>_____ | [ ] sure [ ] unsure [ ]<br>guess |
|-----------------------------|-------------------------------------------|------------------------|----------------------------------|

# Items

1.

An HIM department at a hospital that recently celebrated its 100th anniversary wants to cut release-of-information turnaround time. The team has designed a new request-tracking log, and the director, who is a certified coder, asks what to do before rolling it out to all six sites. Which PDSA step comes next?

- A. Do: test the log at one site on a small scale and collect data.
- B. Study: analyze the results.
- C. Keep planning until the design is perfect.
- D. Act: roll it out to all six sites.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

2.

A quality committee at a 90-bed hospital, chaired by a physician who also teaches at a nearby medical school, tracks the percentage of heart-failure patients who receive written discharge instructions before leaving. The committee meets on the second Tuesday of each month. What type of measure is this?

- A. An outcome measure, because it is reported as a percentage.
- B. A structure measure.
- C. An outcome measure.
- D. A process measure.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

3. [TRANSFER — reversed stem / different variable]

A committee wants an outcome measure for its heart-failure program. Which one qualifies?

- A. The number of committee meetings held.
- B. The percentage of patients given discharge instructions.
- C. The 30-day readmission rate for heart-failure patients.
- D. The number of cardiologists per 100 beds.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

4.

A patient at a surgical hospital with a strong safety record undergoes wrong-site surgery, a sentinel event. The surgeon has 20 years of experience, and the case was scheduled on a holiday weekend. What is the expected next step?

- A. Cancel all holiday-weekend surgeries.
- B. Conduct a root cause analysis focused on system and process causes, then carry out an action plan.
- C. Conduct an FMEA.
- D. Fire the surgeon, since sentinel events are always one person's fault.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

5. [TRANSFER — reversed stem / different variable]

Before launching a new barcode medication process, a team wants to find where it might fail and fix those points in advance. Which tool fits?

- A. A Pareto chart.
- B. Failure mode and effects analysis (FMEA).
- C. A run chart.
- D. Root cause analysis.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

6.

A nurse at a hospital that recently adopted a new EHR skips the barcode scan on a busy night because “everyone does it when the scanner is slow,” and a near miss results. She has a spotless 12-year record. Under Just Culture, what is the appropriate response?

- A. Take no action because of her 12-year record.
- B. Begin disciplinary action for reckless behavior.
- C. Console her, since it was simple human error.
- D. Coach her on the risk and fix the system reasons, such as slow scanners, that invite the shortcut.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**7.** [TRANSFER — reversed stem / different variable]

Under Just Culture, a staff member knowingly disables a required safety alarm, despite clear warnings and no time pressure, to avoid the noise. What is the appropriate response?

- A.** Coach the staff member.
- B.** No action, because no harm occurred.
- C.** Disciplinary action.
- D.** Console the staff member.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**8.**

At a clinic that recently repainted its lobby, patients walk between three desks to register and wait while paper forms are carried by hand between offices. Leadership wants to remove steps that add no value for the patient. The clinic's patient-satisfaction scores are above the national average. Which approach fits best?

- A.** Lean.
- B.** A balanced scorecard.
- C.** Six Sigma DMAIC.
- D.** No change, since satisfaction is above average.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**9.**

A coding manager at a health system that recently added two clinics plots daily discharged-not-final-billed counts on a control chart. For 60 days every point falls inside the control limits with no trends or runs, though a few Mondays ran a little higher. The manager wants to take corrective action on each high day. What does the chart show?

- A.** The two new clinics must be removed from the data.
- B.** Special-cause variation every Monday.
- C.** Common-cause variation in a stable process; improve the system rather than reacting to single days.
- D.** The process is meeting its target.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**10.** [TRANSFER — reversed stem / different variable]

On a stable control chart, one point suddenly falls well above the upper control limit. What does this signal?

- A.** The process is permanently out of control.
- B.** The limits should be recalculated to include the point.
- C.** Common-cause variation.
- D.** Special-cause variation to investigate.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**11.**

A remote coder who joined the team six months ago and works from a home office coded 36 outpatient charts in a 6-hour coding block. The department standard is 5 charts per hour, and her accuracy score was 97%. What percent of the productivity standard did she meet?

- A.** 97%
- B.** 120%
- C.** 6 charts per hour
- D.** 83%

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**12.**

An HIM director at a hospital that recently passed its Joint Commission survey tallies 200 claim holds from last month: missing signatures 110, missing discharge summaries 50, invalid codes 20, other 20. The director has a budget meeting next week. Where should improvement start?

- A.** Missing signatures and missing discharge summaries, which cause 80% of holds.
- B.** Invalid codes, since coding errors are always the biggest risk.
- C.** Spread effort equally across all four categories.
- D.** Wait until after the budget meeting.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

# STOP

Answers and dissection follow. Finish every item and every strip first.

# Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

## 1. Answer A Concept 1

An HIM department at a hospital ~~that recently celebrated its 100th anniversary~~ wants to cut release-of-information turnaround time. The team has designed a new request-tracking log, and the director, ~~who is a certified coder~~, asks what to do before rolling it out to all six sites. Which PDSA step comes next?

**Target word:** next PDSA step **Decides:** The plan is complete; it has not been tested.

|   |                    |                                                              |
|---|--------------------|--------------------------------------------------------------|
| A | <b>Correct</b>     | After planning, test the change on a small scale.            |
| B | Inverted operation | There are no results to study until the test runs.           |
| C | Absolute language  | PDSA relies on quick, small tests rather than perfect plans. |
| D | Dropped element    | Skips testing and studying the results.                      |

## 2. Answer D Concept 2

A quality committee at a 90-bed hospital, ~~chaired by a physician who also teaches at a nearby medical school~~, tracks the percentage of heart-failure patients who receive written discharge instructions before leaving. ~~The committee meets on the second Tuesday of each month.~~ What type of measure is this?

**Target word:** classify the measure **Decides:** It counts a care activity performed.

|   |                   |                                                               |
|---|-------------------|---------------------------------------------------------------|
| A | Camouflage        | Any measure can be a percentage; format does not decide type. |
| B | Adjacent category | Structure covers resources like staffing or equipment.        |
| C | Confusion set     | Outcomes are results in patients, such as readmissions.       |
| D | <b>Correct</b>    | It tracks whether a care step was done.                       |

## 3. Answer C Concept 2 · transfer

A committee wants an outcome measure for its heart-failure program. Which one qualifies?

**Target word:** identify an outcome **Decides:** A result in patient health.

|   |                    |                                                              |
|---|--------------------|--------------------------------------------------------------|
| A | Confusion set      | An activity count about the committee, not a patient result. |
| B | Inverted operation | That is the process measure.                                 |
| C | <b>Correct</b>     | Readmission is a patient result.                             |
| D | Adjacent category  | That is a structure measure.                                 |

## 4. Answer B Concept 3

A patient at a surgical hospital ~~with a strong safety record~~ undergoes wrong-site surgery, a sentinel event. ~~The surgeon has 20 years of experience, and the case was scheduled on a holiday weekend.~~ What is the expected next step?

**Target word:** response to a sentinel event **Decides:** An event has already happened.

|   |                   |                                                                 |
|---|-------------------|-----------------------------------------------------------------|
| A | Camouflage        | Acts on a detail before any analysis shows it mattered.         |
| B | <b>Correct</b>    | Sentinel events call for a thorough RCA and corrective actions. |
| C | Confusion set     | FMEA is prospective, used before a process runs.                |
| D | Absolute language | RCA looks for system causes rather than blaming individuals.    |

### 5. Answer B Concept 3 • transfer

Before launching a new barcode medication process, a team wants to find where it might fail and fix those points in advance. Which tool fits?

**Target word:** prospective tool **Decides:** Nothing has failed yet.

|          |                    |                                                                      |
|----------|--------------------|----------------------------------------------------------------------|
| A        | Adjacent category  | Pareto ranks existing problem counts; there is no data yet.          |
| <b>B</b> | <b>Correct</b>     | FMEA is designed to anticipate failures in a new or changed process. |
| C        | Confusion set      | Run charts track performance over time after a process is running.   |
| D        | Inverted operation | RCA looks back at an event that already happened.                    |

### 6. Answer D Concept 4

A nurse at a hospital ~~that recently adopted a new EHR~~ skips the barcode scan on a busy night because “everyone does it when the scanner is slow,” and a near miss results. ~~She has a spotless 12-year record.~~ Under Just Culture, what is the appropriate response?

**Target word:** classify behavior and response **Decides:** A drifted shortcut she didn’t see as risky: at-risk behavior.

|          |                   |                                                                                                          |
|----------|-------------------|----------------------------------------------------------------------------------------------------------|
| A        | Camouflage        | Past record does not change how the behavior is classified.                                              |
| B        | Adjacent category | Reckless means consciously disregarding a known, unjustified risk; a common drifted shortcut is at-risk. |
| C        | Confusion set     | Human error is an unintended slip; she chose the shortcut.                                               |
| <b>D</b> | <b>Correct</b>    | At-risk behavior is managed by coaching and removing incentives for drift.                               |

### 7. Answer C Concept 4 • transfer

Under Just Culture, a staff member knowingly disables a required safety alarm, despite clear warnings and no time pressure, to avoid the noise. What is the appropriate response?

**Target word:** response to reckless behavior **Decides:** Conscious disregard of a known, substantial risk.

|          |                    |                                                           |
|----------|--------------------|-----------------------------------------------------------|
| A        | Confusion set      | Coaching fits at-risk drift, not conscious disregard.     |
| B        | Dropped element    | Just Culture judges the behavior, not the outcome.        |
| <b>C</b> | <b>Correct</b>     | Reckless behavior warrants discipline under Just Culture. |
| D        | Inverted operation | Consoling is for unintended human error.                  |

### 8. Answer A Concept 5

At a clinic ~~that recently repainted its lobby~~, patients walk between three desks to register and wait while paper forms are carried by hand between offices. Leadership wants to remove steps that add no value for the patient. ~~The clinic’s patient satisfaction scores are above the national average.~~ Which approach fits best?

**Target word:** choose the improvement method **Decides:** Waste in motion, waiting, and handoffs.

|          |                   |                                                                         |
|----------|-------------------|-------------------------------------------------------------------------|
| <b>A</b> | <b>Correct</b>    | Lean targets waste and non-value-added steps.                           |
| B        | Adjacent category | A scorecard reports strategic measures; it does not redesign a process. |
| C        | Confusion set     | Six Sigma targets variation and defect rates.                           |
| D        | Camouflage        | Good scores do not remove waste in the process.                         |

## 9. Answer C Concept 6

A coding manager at a health system ~~that recently added two clinics~~ plots daily discharged-not-final-billed counts on a control chart. For 60 days every point falls inside the control limits with no trends or runs, though a few Mondays ran a little higher. The manager wants to take corrective action on each high day. What does the chart show?

**Target word:** interpret a stable chart **Decides:** All points within limits, no patterns.

|          |                   |                                                                         |
|----------|-------------------|-------------------------------------------------------------------------|
| A        | Camouflage        | Nothing on the chart points to the new clinics.                         |
| B        | Confusion set     | Points inside the limits without a qualifying pattern are common cause. |
| <b>C</b> | <b>Correct</b>    | Reacting to common-cause points (tampering) adds variation.             |
| D        | Adjacent category | Stable is not the same as capable; the target may still be missed.      |

## 10. Answer D Concept 6 • transfer

On a stable control chart, one point suddenly falls well above the upper control limit. What does this signal?

**Target word:** interpret an out-of-limit point **Decides:** A point beyond the control limit.

|          |                    |                                                              |
|----------|--------------------|--------------------------------------------------------------|
| A        | Absolute language  | One signal calls for investigation, not a permanent verdict. |
| B        | Dropped element    | Skips investigating the signal before changing the chart.    |
| C        | Inverted operation | Common cause stays within the limits.                        |
| <b>D</b> | <b>Correct</b>     | A point outside the limits signals an assignable cause.      |

## 11. Answer B Concept 7

A remote coder ~~who joined the team six months ago and works from a home office~~ coded 36 outpatient charts in a 6-hour coding block. The department standard is 5 charts per hour, ~~and her accuracy score was 97%~~. What percent of the productivity standard did she meet?

**Target word:** percent of standard **Decides:**  $36 / 6 = 6$  charts per hour vs. 5.

|          |                    |                                               |
|----------|--------------------|-----------------------------------------------|
| A        | Camouflage         | That is her accuracy score, not productivity. |
| <b>B</b> | <b>Correct</b>     | $6 / 5 = 1.20 = 120\%$ of standard.           |
| C        | Dropped element    | Her rate, but not compared to the standard.   |
| D        | Inverted operation | Divides the standard by her rate.             |

## 12. Answer A Concept 8

An HIM director at a hospital ~~that recently passed its Joint Commission survey~~ tallies 200 claim holds from last month: missing signatures 110, missing discharge summaries 50, invalid codes 20, other 20. ~~The director has a budget meeting next week.~~ Where should improvement start?

**Target word:** prioritize with Pareto **Decides:** Two categories make up 160 of 200 holds.

|          |                   |                                                      |
|----------|-------------------|------------------------------------------------------|
| <b>A</b> | <b>Correct</b>    | $(110 + 50) / 200 = 80\%$ ; these are the vital few. |
| B        | Absolute language | The counts show coding is a small share here.        |
| C        | Dropped element   | Ignores the frequency ranking.                       |
| D        | Camouflage        | The meeting has nothing to do with where to focus.   |

## Triage grid

Write each item number in the box it belongs to.

|                       | Right                                                 | Wrong                                                                                 |
|-----------------------|-------------------------------------------------------|---------------------------------------------------------------------------------------|
| <b>Sure</b>           | Owned.                                                | <b>Concept inversion — top priority.</b> A wrong mental model; it survives rewording. |
| <b>Unsure / guess</b> | Recognition, not mastery. Re-drill with the transfer. | Not learned yet.                                                                      |

## Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

| Trap               | What it does                                                 | In this drill | Your misses |
|--------------------|--------------------------------------------------------------|---------------|-------------|
| Dropped element    | Leaves out a required term or step.                          | 5             |             |
| Wrong stem number  | A figure copied from the prompt that is not the answer.      | 0             |             |
| Adjacent category  | The right idea in the wrong bucket.                          | 6             |             |
| Inverted operation | Direction flipped: add/subtract, cause/effect, higher/lower. | 6             |             |
| Absolute language  | Always, never, guarantee — where judgment applies.           | 4             |             |
| Confusion set      | Every option is real; the test is exact role-matching.       | 8             |             |
| Camouflage         | Answers a stem fact that was never the question.             | 7             |             |

## Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

| # | Concept                                         | Rule                                                                                                                                                                                         | Trap                                                                          | Tell                                                                         |
|---|-------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------|------------------------------------------------------------------------------|
| 1 | <b>PDSA cycle</b>                               | Plan, Do (test small), Study (compare results to prediction), Act (adopt, adapt, or abandon) — test before spreading.                                                                        | Rolling a change out everywhere before a small test.                          | A change has just been designed and the question asks what is next.          |
| 2 | <b>Structure, process, and outcome measures</b> | Structure = resources and setting; process = what is done to or for patients; outcome = the result in patient health.                                                                        | Calling a care-delivery percentage an outcome.                                | “Percentage of patients who received...” vs. “rate of...” results.           |
| 3 | <b>RCA vs. FMEA</b>                             | Root cause analysis looks back at an event to find system causes; FMEA looks ahead at a process to find and fix failure points before harm.                                                  | Using RCA for a process not yet launched, or blaming individuals.             | “After a sentinel event” vs. “before launching.”                             |
| 4 | <b>Just Culture responses</b>                   | Human error → console; at-risk behavior (drifting into a shortcut) → coach and fix system reasons; reckless behavior (conscious disregard of substantial risk) → discipline.                 | Punishing every error, or excusing reckless choices because of a good record. | Whether the person chose the risk, and whether they knew it was unjustified. |
| 5 | <b>Lean vs. Six Sigma</b>                       | Lean removes waste and non-value steps to improve flow; Six Sigma reduces variation and defects using DMAIC.                                                                                 | Choosing Six Sigma for a flow and waste problem.                              | Walking, waiting, handoffs (Lean) vs. inconsistent results (Six Sigma).      |
| 6 | <b>Common- vs. special-cause variation</b>      | Points within control limits with no pattern show common-cause variation — change the system, don’t react to single points; a point outside the limits signals special cause to investigate. | Reacting to every high point, or treating “in control” as “meeting target.”   | A control chart and a manager who wants to act on each day.                  |
| 7 | <b>Measurable performance standards</b>         | Employee performance is judged against clear, measurable standards; percent of standard = actual rate / standard rate.                                                                       | Mixing quality and productivity figures, or inverting the ratio.              | An output count, time worked, and a per-hour standard.                       |
| 8 | <b>Pareto principle</b>                         | Rank causes by frequency and focus first on the vital few that create most of the problem.                                                                                                   | Spreading effort evenly or starting with the easiest category.                | A tally of causes by count.                                                  |

**Course scope (public description):** Quality and Performance Management and Methods examines quality initiatives within healthcare. Quality issues cover human resource management, employee performance, and patient safety. This course focuses on quality improvement initiatives and performance improvement with the health information management perspective.